Baylor Scott & White Health
Austin Southwest
Family Medicine
9521 W US Highway 290 Suite 105
Austin, TX 78736
PH: (512) 654-4300 | FAX: (512) 654-4301

Our office will send last office visit note upon request. If requesting additional records, please contact our medical records department:
PH: (512) 509-9074
Fax: (844) 331-3877

BaylorScott&White HEALTH

TO: Pam Heath
FROM: Baylor Scott & White Austin Southwest Clinic
FAX: (512) 295-7070
PHONE: (512) 654-4300
PHONE: (512) 295-7000
FAX: (512) 654-4301
PAGES: 9 pages
DATE: 3-2-2026
RE: Yolanda Villarreal DOB: 11-06-1944

☐ Urgent ☐ For Review ☐ Please Comment ☐ Please Reply ☐ Please Recycle

Comments:

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Thank you.

PAM HEALTH AT HOME

Fax

PAM - Please Note
PCP: Jacqueline Stritzke

To: CASAS, CHEYANNE **From:** Pam Health at Home - Texas

Fax: 15126544601 **Pages:** 8

Phone: **Date:** 02-24-2026 3:00 PM

Re: Order(s) for Signature

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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Order No:485-25817

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
12025171501	02/16/2026	02/16/2026 - 04/16/2026	9589	262009456

6. Patient's Name and Address
Villarreal, Yolanda P (9589)
509 Voyager
Kyle, TX 78640
Phone: (512) 422-3516

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 11/06/1944 **9. Sex:** ☐ M ☑ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Advil 200 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1-2	daily as needed	oral	LS - PRN/As Needed
Alendronate 70 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every 7 days	oral	LS
Benzonatate 200 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1	3 times a day as needed	oral	LS - PRN/As Needed
Calcium 600 mg (as carbonate)-vit D3 10 mcg (400 unit) chewable tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Cholecalciferol (vitamin D3) 25 mcg (1,000 unit) capsule [Amount Of/ Dosage Form: 0 Capsule]	1	daily	oral	LS
Latanoprost 0.005 % eye drops [Amount Of/ Dosage Form: 0 Metric Drop]	1	each eye every PM	ophthalmic (eye)	LS
Losartan 50 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Omega 3-dha-epa-fish oil 300 mg-1,000 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1	daily	oral	LS
Omeprazole 20 mg capsule,delayed release [Amount Of/ Dosage Form: 0 Capsule]	1	daily	oral	LS
Pravastatin 20 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every PM	oral	LS
Turmeric root extract 150 mg-ginger root extract 25 mg chewable tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Zinc 50 mg (as gluconate) tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
G31.84	Mild cognitive impairment of uncertain or unknown etiology		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

Medicare#:

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13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
M19.012	Primary osteoarthritis, left shoulder		
I10	Essential (primary) hypertension		
K21.9	Gastro-esophageal reflux disease without esophagitis		
E78.5	Hyperlipidemia, unspecified		
H40.9	Unspecified glaucoma		
Z91.81	History of falling		

14. DME and Supplies:
DME: Cane, Tub/Shower Bench

15. Safety Measures:
8 - 24 hour Supervision, 911 for Emergency, Assistance with IADLs, Emergency Plan Developed, Fall / Safety Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Medication Safety / Storage, Safety in ADLs, Slow Position Change, Standard Precautions/Infection Control, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
No Added Salt,
Heart Healthy

17. Allergies:
No Known Allergies

18.A. Functional Limitations
☐ 1- Amputation ☑ 4- Hearing ☑ 7- Ambulation
☑ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☑ B- Other (Specify): pain

18.B. Activities Permitted
☐ 1- Complete Bedrest ☐ 5- Exercises Prescribed ☑ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☐ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☑ D- Other (Specify):SBA

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

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20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☒ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:

Frequency
SN Frequency: 1W2 Starting Week of 02/16/2026,PT Frequency: 2W3 Starting Week of 02/16/2026,

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 159 or > 169
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:
- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:
- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:
- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

GENITOURINARY:
- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.

HOSPITALIZATION RISK FACTORS:
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:
- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity,

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managing stress, avoiding high sodium foods, and monitor blood pressures at home.

• SN to instruct patient/caregiver on Heart Healthy diet.

PT

PHYSICAL THERAPY:

• Ther ex, Transfer training, Gait training, Bed mobility training, Balance activities, Endurance training, AD management, Heat/Cold modalities, Manual therapy, Pt/Caregiver education, Monitor vitals/Pain level; all listed interventions PRN

• PT/A to instruct the patient/caregiver to contact agency to report any fall with or without minor injury. Call 911 for fall resulting in serious injury or causing severe pain or immobility

PT/A to instruct patient to change positions slowly

PT/A to instruct the Patient/caregiver to remove clutter from patient's path such as clothes, books, shoes, electrical cords, or other items that may cause patient to trip

Assessment Summary

81 Y/O Spanish speaking female Hx HTN, glaucoma, HLD, GERD, admitted to PAM health at home HH per referral from PCP after a fall with resulting pain Lt humerus. She had Xry done which was negative for a fracture but did show osteoarthritis in the AC joint. Pt lives alone, but lately has been staying with her 2 daughters alternating between homes due to need for increase assistance in ADL, IADL. Pt is alert and oriented, pleasant and cooperative. Lung sounds are clear, HR regular, trace edema RLE and 1+ LLE. She has dizziness at times but denied dizziness as cause for her fall. Skin is intact, abdomen soft. She is continent of bowel and bladder with some urinary frequency and occasional stress incontinence. Pt reports good appetite and hydration. Pt C/O pain Lt arm and also intermittently has back and knee pain. She takes advil as needed. Pt ambulatory without AD in the home and cane for community. Pt is homebound due to weakness, pain, decreased balance and high fall risk requiring assistance and use of AD to leave home. Pt will benefit from SN for assessment/teaching related to medications, pain management, HTN, safety and fall prevention. Pt. needs PT eval/tx for mobility, transfers, gait, balance, endurance, ADL and establish HEP. Pt goal is to regain her independence and eventually return to her home.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. Villarreal, Yolanda P is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)

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c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 02.10.2026 BY CHEYANNE MARIE CASAS, MD and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 2.16.2026.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Decline in mental, emotional, or behavioral status in the past 3 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN,PT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); ADL Assistance Needed (M2102 & M2110) ; Home Safety Risks

Psychosocial Assessment

Primary Language -Spanish;
Sleep Rest:Inadequate,Explain-wakes up at night and can't go back to sleep;

Cognitive Status

Cognitive status: Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required; Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions

Advance Directive

Patient/Caregiver reports no Advanced Directives in place
Comments -: Sandra Mendez representative
Patient does not have an advance directive;
Have an Advanced Care Plan and it is documented in the medical record: No - Unable to provide;
Have a Surrogate Decision Maker and information documented in the medical record: ;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Unable to safely leave home unassisted; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes

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- Comments: Pt requires assistance and use of AD related to weakness and pain.

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services
Discharge to care of physician
Discharge to caregiver
Discharge when goals met

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG

- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG

- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG

- LTG-Patient/caregiver will demonstrate decreased pain with ambulation/activity as evidenced by increased activity tolerance and pain < 4/10 by end of episode.

GENITOURINARY: - LTG

- LTG-Patient will tolerate collection of urine without pain and discomfort dle s/sx UTI by end of episode.

HOSPITALIZATION RISK FACTORS: - LTG

- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG

- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

MOBILITY: - LTG

- Pt will exhibit L shoulder flexion >120deg, in order to improve ADL performance and improve LUE mobility, in 3 weeks.

MOBILITY/GAIT: - LTG

- Pt will ambulate x150' (total distance) over level surfaces at mod I level, or with SUP, using LRD (PRN), in order to safely access different areas of the home, as needed, in 3 weeks.
- The patient will be free from Falls/injury during the PT level of care, in 3 weeks.

MOBILITY/GAIT: - STG

- Pt will perform sit<>stand transfers at indep, or mod I level, in order to safely stand from home furniture, in 2 weeks.

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23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/16/2026
Signed By: Byrnes, Sarah - RN on
02/16/2026 05:44 PM, Cosigned by
Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
Cheyanne Casas MD
4036 Cromwell Dr
KYLE, TX 78640,NPI: 1528047693, License#: P4636,Phone:
(512) 654-4600, Fax: (512) 654-4601
Jacqueline Stritz M.D.
RSW - BUA

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:
3-2-26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.